

Radiotherapy for advanced prostate cancer

Radiotherapy is most often used to shrink prostate cancer that has spread to the bones.

On this page

- [What is radiotherapy for advanced prostate cancer?](#)
- [Palliative radiotherapy](#)
- [Planning radiotherapy for advanced prostate cancer](#)
- [Having radiotherapy for advanced prostate cancer](#)
- [Side effects of radiotherapy for prostate cancer](#)
- [How we can help](#)

What is radiotherapy for advanced prostate cancer?

[Radiotherapy](#) uses high-energy rays to treat cancer. It destroys cancer cells in the area where it is given. It can be given in different situations to treat [advanced \(metastatic\) prostate cancer](#).

You have external beam radiotherapy for advanced prostate cancer. This means it is given from a machine outside the body (externally).

If you are having radiotherapy, you will see a clinical oncologist and a specialist therapeutic radiographer. A clinical oncologist is a cancer doctor who specialises in radiotherapy, chemotherapy and other drugs to treat cancer. Therapeutic radiographers are experts in radiotherapy and are specially trained in cancer treatment. They can also give you support, advice and information about your radiotherapy.

We have separate information about having [radioisotope therapy for advanced prostate cancer](#).

Radiotherapy to the prostate gland

If you have been newly diagnosed with advanced prostate cancer, your cancer team might talk to you about radiotherapy to the prostate gland. Research has shown that treating the prostate can help with symptoms, even when the cancer has spread to other areas. It can also help some people live longer. But radiotherapy to the prostate is not suitable for everyone.

You usually have radiotherapy once a day, Monday to Friday, over 4 weeks. Sometimes you have it once a week for 5 to 6 weeks.

Palliative radiotherapy

When radiotherapy is used to relieve symptoms, it is called palliative radiotherapy.

Radiotherapy for advanced prostate cancer is most often used to shrink cancer that has spread to the bones.

It can strengthen the bone and reduce pain. This helps to keep you as active as possible. Sometimes treatment to the bones may also help to control the cancer.

Radiotherapy can also help to treat:

- pain from the cancer in the prostate
- pain if the cancer is in a [lymph node](#) or pressing on the bowel (rectum)
- blood in the urine (pee).

Radiotherapy for metastatic spinal cord compression

Sometimes cancer in the bones of the spine grows and presses on the spinal cord. This may stop the spinal cord nerves from working properly. This is called [metastatic spinal cord compression](#) or MSCC.

MSCC can be treated with external beam radiotherapy to the spine. You usually have a single treatment. Some people may have surgery or cancer drugs to treat MSCC. You may also be given steroids to relieve the compression.

Planning radiotherapy for advanced prostate cancer

Your cancer doctor plans your radiotherapy carefully. This is to make sure it is as effective as possible, while causing as few side effects as possible.

You will have a hospital appointment to plan your treatment. During the [planning visit](#), you will have a [CT scan](#). The scan is to help your radiotherapy team decide on the dose and area to treat. The hospital will send you information if you need to prepare for your scan.

During the scan, you need to lie still in the same position you will be in for your radiotherapy.

The person who gives you radiotherapy is called a therapeutic radiographer. They may make some permanent marks (tattoos) the size of a pinpoint on your skin. These are used to make sure you are in the correct position for your treatment. They also show where the radiotherapy machine will direct its rays. The marks are only made with your permission. It may be a little uncomfortable. If you are worried about this, talk to the radiographer.

If you are having radiotherapy to the prostate

Before your CT scan, you may need to have a special diet or take medicine to empty your bowel. You may also need to drink water to fill your bladder. At some hospitals, you may be asked to go to the toilet so your bladder is empty. Doing these things helps get very clear CT scan pictures to help plan your treatment.

You may have a small amount of liquid passed into your rectum. This is called an enema. It helps you empty your bowel fully when you go to the toilet. You can usually do this yourself. If you need help, speak to your radiotherapy team.

You may be advised to start [pelvic floor exercises](#). Doing these exercises before, during and after your radiotherapy can help with bladder side effects.

Having radiotherapy for advanced prostate cancer

You usually have external beam radiotherapy in a hospital radiotherapy department as an outpatient. The radiotherapy machine looks like a big x-ray machine. It is called a linear accelerator or LINAC.

You may have external beam radiotherapy as a single treatment or as a series of short, daily or, weekly treatments called fractions. If you are having multiple treatments, they are given from Monday to Friday, with a break at the weekend.

At the start of a treatment session, the therapeutic radiographer will make sure you are in the correct position on the couch and that you are comfortable. Radiotherapy is not painful. But you must lie still during the treatment. If you are planning to take painkillers, you may want to take them before you have treatment.

When everything is ready, the therapeutic radiographer leaves the room so you can have the radiotherapy. The treatment only takes a few minutes. You can talk to them through an intercom or signal to them during the treatment. They can see and hear you from the next room.

Your clinical oncologist, specialist nurse or therapeutic radiographer will explain your treatment and its possible side effects. They can give you advice on what you can do to manage any side effects. They can also help answer any questions you may have.

Side effects of radiotherapy for prostate cancer

Side effects of radiotherapy to the prostate

Side effects usually build up slowly after you start treatment. They may continue to get worse for a couple of weeks after treatment. But after this, most side effects improve gradually over the next few weeks.

Your radiotherapy team will talk to you about side effects. They will explain what to expect and give you advice on what you can do to manage them.

Always tell them about your side effects rather than trying to treat them yourself. There are usually things they can do to help.

We have more detailed information about side effects of external beam [radiotherapy to the prostate](#) and [pelvic radiotherapy](#).

Side effects of palliative radiotherapy

The side effects of palliative radiotherapy are usually mild. They will depend on the area being treated. Always tell your cancer team about any side effects. There are usually ways to treat or manage them.

[Feeling very tired](#) is a common side effect. This should gradually improve a few weeks after treatment finishes. Try to pace yourself and plan your day so you have time to rest. Gentle exercise, like short walks, can give you more energy.

You may [feel sick](#) if the area treated is close to your tummy – for example, the ribs or spine. Your doctor can give you anti-sickness drugs to help prevent or control sickness. If you only have 1 or 2 treatment sessions, you may have flu-like symptoms for a few days afterwards.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Below is a sample of the sources used in our prostate cancer information. If you would like more information about the sources we use, please contact us at informationproductionteam@macmillan.org.uk

Prostate cancer: diagnosis and management. NICE guideline [NG131] Published: 09 May 2019 Last updated: 15 December 2021 (accessed October 2024) <https://www.nice.org.uk/guidance/ng131>

Parker, C. et al. Prostate cancer: ESMO Clinical Practice Guidelines for diagnosis, treatment and follow-up. Annals of Oncology, Volume 31, Issue 9, 1119–1134 (accessed October 2024) [https://www.annalsofoncology.org/article/S0923-7534\(20\)39898-7/fulltext#articleInformation](https://www.annalsofoncology.org/article/S0923-7534(20)39898-7/fulltext#articleInformation)

[Cancer Information Development team](#)

Author

[Dr Ursula McGovern](#)

Reviewer

Consultant Medical Oncologist & Honorary Associate Professor
University College Hospitals, London

Date reviewed

Reviewed: 01 February 2025 | Next review: 01 February 2028

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help